

'Well-led': an evidence review

September 2025



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Contents

Acknowledgements	3
Acronyms and notes on terminology	3
1. Introduction	5
2. Why did we undertake this evidence review?	6
3. What question did we seek to answer?	8
4. What was the scope of this evidence review?	9
5. How did we undertake the review?	11
6. About the findings	11
7. The findings	15
8. What do we suggest should happen next?	21
Bibliography	22

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The views expressed in this publication are those of the author(s) and not necessarily those of the NIHR SSCR, the National Institute for Health and Care Research, or the Department of Health and Social Care.

Acronyms and notes on terminology

CQC	Care Quality Commission
DASS	Director of Adult Social Services
DHSC	Department of Health and Social Care
LGA	Local Government Association
NCAG	National Co-production Advisory Group
NIHR	National Institute for Health and Care Research
AF	Assessment Framework
SCIE	Social Care Institute for Excellence
SSCR	School for Social Care Research
TLAP	Think Local Act Personal

- ‘Family and friend carers’ is used to describe people who provide care and support to family or friends, unpaid, whether that’s due to illness, disability, a mental health need, addiction, or something else.

- 'Care workers' is used to describe people who are employed to provide care and support to adults. This predominantly refers to people who are paid to provide care and support by virtue of a contract of employment (or any other contract), but it may include volunteers who provide care and support as part of their work for a voluntary organisation.
- 'Care' and 'social care' are used to describe not only the care and support which is provided or arranged by the state through councils, but also that which people arrange and fund themselves.
- 'The frontline' workforce is sometimes used to describe the elements of the adult social care workforce that are people-facing and/or practice-focused. This is a well-used term, but many dislike its 'war-like' nature and the connotation the workforce is confronting an enemy. We have not used 'frontline' in this report, instead favouring 'person-facing'.

1. Introduction

Think Local Act Personal (TLAP), the National Co-production Advisory Group (NCAG), and Chloë Reeves (an independent researcher) worked together to review the available evidence around what it feels like to experience 'well-led' health and/or care services and organisations. This report explains how and why we did this, what we found, and what we suggest should happen next.

This work was undertaken between July and December 2023. It was mostly funded by a NIHR SSCr Career Development Award¹ awarded to Chloë to help build research capacity within social care. Chloë is hosted by the NIHR Health and Social Care Workforce Research Unit at King's College London.²

- Think Local Act Personal³ is a network of people and organisations working to make care and support more personalised, so that everyone can live life their way. Everything it does is built upon the experiences of people who draw on care and support, based on the belief that no decision about people should ever be made without them. The network includes individuals who draw on care and support, carers, groups, providers, community organisations, local and national government bodies, and the NHS.
- The National Co-production Advisory Group (NCAG)⁴ is a membership group made up of people who draw on care and support and family and friend carers. NCAG members are committed to personalised care and support because they know from experience that they can improve wellbeing and lead to improved health outcomes.

Members promote personalised care and support at a national, regional, and local level. They partner with TLAP to make sure it's work is fully co-produced. They also offer organisations help with Making It Real for delivering good, coordinated and personalised care and support, as well as development around co-producing.

- Chloë Reeves⁵ is an independent researcher. She chose to work in adult social care because of her experiences as a carer. She's held various policy, practice, research and campaigning roles in local government and the charity sector. She has worked freelance for eight of her 19 years in social care, mostly on policy analysis,

1 For more information about NIHR School of Social Care Research awards, please visit:

www.sscr.nihr.ac.uk/capacity

2 For more information about the NIHR Health and Social Care Workforce Research, please visit:

www.kcl.ac.uk/hscwru

3 For more information about TLAP, please visit: www.thinklocalactpersonal.org.uk

4 For more information about NCAG, please visit:

www.thinklocalactpersonal.org.uk/Browse/Co-production/National_Co-production_Advisory_Group/

5 For more information about Chloë's previous research, please visit:

<https://orcid.org/0009-0000-0055-1417>

advocacy campaigns, and qualitative research projects. Chloë has previously partnered with TLAP and NCAG in her role as a National Voices Associate.

2. Why did we undertake this evidence review?

This evidence review sought to achieve two distinct but coherent aims:

- a. To support TLAP's work to update Making It Real with 'well-led' I statements.
- b. To inform the development of a PhD proposal to understand and assure the pipeline of Directors of Adult Social Services (DASSs).

2a. TLAP's work to update Making It Real

'Making It Real' is a framework and set of statements that describe what good, coordinated and personalised care and support look like from the perspective of people drawing on it (TLAP, 2018). Making It Real is built around six themes to reflect the most important elements of personalised care and support. Each theme has several I statements that describe what good looks like from an individual perspective. These are followed by We statements that express what organisations should be doing to make sure people's actual experience lives up to the I statements (TLAP, 2018).

TLAP, NCAG and National Voices⁶ were asked by colleagues at the Care Quality Commission (CQC) to support their work to incorporate Making It Real into CQC's Assessment Framework (AF)⁷. The Assessment Framework is structured around CQC's five key questions⁸ of the services, organisations and public bodies that come under their regulation:

1. Are they safe?
2. Are they effective?
3. Are they caring?
4. Are they responsive to people's needs?
5. Are they well-led?

⁶ National Voices is a leading coalition of health and social care charities in England. They have more than 200 members covering a diverse range of health conditions and communities, connecting them with the experiences of millions of people. They work together to strengthen the voice of people: people drawing on health and care, carers, their families, and the voluntary organisations that work for them.

⁷ For more information about the CQC's Single Assessment Framework, please visit:

www.cqc.org.uk/news/our-new-single-assessment-framework

⁸ For more information about the CQC's five key questions, please visit:

www.cqc.org.uk/about-us/how-we-do-our-job/five-key-questions-we-ask

TLAP, NCAG and National Voices found a strong alignment between Making It Real and CQC's first four quality statements, but identified a gap in the 'well-led' theme.

As shown below, Making It Real recognises 'the people who support me' as one of the six themes of personalised care and support.

The graphic is titled 'Workforce' in white text on an orange background. Below the title, on a dark blue background, are two white rounded rectangles. The first rectangle is titled 'I Statements' and contains three statements: 'I am supported by people who see me as a unique person with strengths, abilities and aspirations.', 'I am supported by people who listen carefully so they know what matters to me and how to support me to live the life I want.', and 'I am supported to make decisions by people who see things from my point of view, with concern for what matters to me, my wellbeing and health.' The second rectangle is titled 'We Statements' and contains three statements: 'We don't make assumptions about what people can or cannot do and don't limit or restrict people's options.', 'We see people as individuals with unique strengths, abilities, aspirations and requirements and value people's unique backgrounds and cultures.', and 'We have a 'can do' approach which focuses on what matters to people and we think and act creatively to make things happen for them.' The third statement in the 'We Statements' section is: 'We keep up to date with local activities, events, groups and learning opportunities and share this knowledge so that people have the chance to be part of the local community.'

Workforce

I Statements

I am supported by people who see me as a unique person with strengths, abilities and aspirations.

I am supported by people who listen carefully so they know what matters to me and how to support me to live the life I want.

I am supported to make decisions by people who see things from my point of view, with concern for what matters to me, my wellbeing and health.

I have considerate support delivered by competent people.

We Statements

We don't make assumptions about what people can or cannot do and don't limit or restrict people's options.

We see people as individuals with unique strengths, abilities, aspirations and requirements and value people's unique backgrounds and cultures.

We know how to have conversations with people that explore what matters most to them – how they can achieve their goals, where and how they live, and how they can manage their health, keep safe and be part of the local community.

We have a 'can do' approach which focuses on what matters to people and we think and act creatively to make things happen for them.

We keep up to date with local activities, events, groups and learning opportunities and share this knowledge so that people have the chance to be part of the local community.

Extract from: TLAP (2023). *Making It Real Guide*

However, Making It Real does not specifically identify what 'well-led' care and support looks like.

Meeting the requirements of the Assessment Framework depends on good leadership and the commitment and enthusiasm of the health and care workforce to realise the vision of Making It Real. For that reason, TLAP, NCAG and National Voices recommended further work be co-produced with people with lived experience and representatives of the

workforce to describe what ‘well-led’ care and support looks like.

TLAP is working alongside CQC to facilitate the co-production of this extension to Making It Real and into the Assessment Framework. This evidence review is informing co-production workshops and support discussion by introducing what we already know about what people drawing on services, family and friend carers, and members of the workforce say experiencing well-led care and support looks like.

It is also important to consider where understanding is limited. Crucially, there has been a historic lack of data about what people want from social care leadership especially when it comes to equity, diversity, and inclusion. We are pleased that new research from Skills for Care, expected in 2025, begins to offer deeper insight into who leads, how inclusive leadership feels to the workforce, and how we can ensure leadership better reflects the diversity of the communities we serve.

Making It Real is a widely accepted vision of what good, coordinated and personalised care and support looks like. Through this evidence review, we aim to understand if it sufficiently describes what ‘well-led’ care and support looks like, or if there are further considerations that our research will need to make to ensure it centres people drawing on care and support and family and friend carers.

3. What question did we seek to answer?

In the early stages of thinking about our two projects, we co-produced the following research question for a ‘well-led’ evidence review:

Q. What has already been published about what experiencing a well-led health and/or care service or organisation looks like from the perspective of:

- a. people drawing on health, care and support services,*
- b. family and friend carers, and*
- c. members of the adult social care and health workforce?*

At a workshop, we then discussed, agreed and clarified this question by agreeing:

- ‘Family and friend carers’ includes young carers of adults.
- People with caring responsibilities do not always recognise themselves to be carers, and some people with caring responsibilities prefer not to be identified as a ‘carer’. The review will be sensitive to these and related issues.
- ‘Members of the adult social care and health workforce’ includes volunteers.

- 'Members of the adult social care and health workforce' includes people in elected but unpaid/nominally paid roles, including Councillors, Non-executive Directors, and Board Members / Trustees of provider organisations.
- The aim of this evidence review is to capture what has been written about experiences of well-led services and organisations and not to collate evidence of different leadership models.

4. What was the scope of this evidence review?

4a. Adult care and health services and organisations

We considered focusing on those services that fall under the regulatory remit of the CQC. The CQC "ensures the quality and safety of care in hospitals, dentists, ambulances, and care homes, and the care given in people's own homes" (HM Government, 2023). However, we felt this to be a limited definition that would exclude some community-based services and facilities.

We also noted that the CQC has regulated NHS trusts for some time and has now extended into the regulation of council provided adult social care services and Integrated Care Systems. Council duties under the Care Act 2014 extend much further than formal, regulated services. These duties include ensuring the provision of preventative services, market shaping, providing information and advice (including signposting), and the overarching duty to promote wellbeing.

We therefore agreed this evidence review should consider care and health services and organisations in the broadest sense.

Specific clarificatory points identified by TLAP and NCAG included:

- Supported living is not a service regulated by CQC, but the leadership within supported living is critical to people's experiences and outcomes of these settings. (Supported housing does fall under local authorities' broader Care Act duties).
- Service models like the Dutch-pioneered 'Buurtzorg' model⁹ have sometimes been excluded by CQC's traditional, hierarchical ideas of being 'well-led'.

⁹ For more information about Buurtzorg, please visit: www.buurtzorg.com

- The perspectives of people employing Personal Assistants (PAs) and of people working as PAs are explicitly within the scope of this evidence review.

We agreed it may be possible to learn from services and organisations that are not care and health focused. We noted that leadership training in care and health sometimes draws on resources that are not specific to care and health. However, we agreed that to keep this review focussed, we would start with care and health and only broaden out if we had time to do so.

4b. Types of evidence

We kept this broad: research of all study designs, reports, articles, case studies, policies, guidance, podcasts, recorded interviews and so on. We took this decision because our experience points to much of the available evidence being 'grey literature', i.e., publications outside of academic and commercial publishing.

4c. Date of publication

TLAP have used the findings of this review to guide its own work and development of well-led as part of the partnership with the CQC. The project began in January 2025, and this review will be published as TLAP continue to work to co-produce new Making It Real 'well-led' content.

4d. Location of publication

This review has focussed upon:

1. Publications from England.
2. Publications from UK devolved nations.
3. Publications from other countries, using the King's Fund's work on comparable countries and translatable / transferrable learning as a guide to prioritisation.¹⁰

¹⁰ For more information about comparing health and care systems, please visit: www.kingsfund.org.uk/publications/nhs-compare-health-care-systems-other-countries

5. How did we undertake the review?

Jan 2023	Drafted our research question.
Jan–Jun 2023	Secured funding.
Jul 2023	Reconfirmed commitment to the review and agreed general approach.
Aug 2023	Half-day co-production workshop to clarify research question and agree scope.
Aug–Sep 2023	Desktop research: identifying and selecting contributions; extracting data; collating data; and [manual] thematic analysis.
Sep 2023	Half-day co-production workshop to consider and, where appropriate, validate interim findings and themes, as well as to consider gaps in the available publications.
Sep–Nov 2023	Further desktop research: identifying and selecting additional contributions; extracting data; collating data; and [manual] thematic analysis – informed by the advice given and decisions made at the September co-production workshop.
Nov 2023	Half-day co-production workshop to consider and, where appropriate, validate updated findings, plus develop recommendations.
Nov–Dec 2023	Refined findings and recommendations. Finalised report.

6. About the findings

These fall into two categories:

- a. Findings about the evidence.
- b. Findings about what well-led services and organisations look like
 - Caveats and context
 - Findings (of which we recommend further discussion).

6a. Findings about the evidence

We had originally intended this evidence review to be akin to a rapid review¹¹ rather than a scoping review¹². TLAP and NCAG suspected there were gaps in the evidence, but none of us realised how pronounced these gaps are. We made several important findings about the evidence itself and these, as explained below in Section 5b, have had a significant impact on our findings about what well-led services and organisations look like.

- The term 'well-led' is rarely used in publications beyond those authored by CQC. While search terms like 'leadership' wielded some results, they tended to be about leadership theories and models rather than people's experiences of services and organisations.
- While desktop searches wielded some results, many of the identified contributions were submitted by TLAP members, NCAG members, and their respective networks.
- Selecting relevant contributions took significant time, as experiential evidence was often buried amidst discussion of leadership theory, leadership models, and considerations of business management and governance.
- Most of the published evidence is from the perspective of people in positions of leadership / power within health and social care. Some of these people are practice leaders and some are 'thought leaders'.
- There is some published evidence from the perspective of people in person-facing roles.
- There is very little published evidence from the perspective of people drawing on services and family and friend carers.

6b. Findings about what well-led services and organisations look like

Some caveats and context:

These findings are not presented in any sort of hierarchy or priority order. As described above in Section 5a, we found the balance of available evidence to be heavily weighted toward people within the care and health workforce who hold positions of leadership and power. There is relevant and useful learning within these contributions. However, there is also a risk that a synthesis of evidence amplifies these voices while inadvertently diminishing the perspectives of people drawing on services and family and friend carers.

¹¹ For more information about rapid reviews, please visit:

<https://academic.oup.com/eurjcn/article/20/5/515/6278029>

¹² For more information about scoping reviews, please visit:

www.tandfonline.com/doi/abs/10.1080/1364557032000119616

We see these findings as points for further discussion rather than definitive conclusions.

Due to the gaps in evidence noted above, we do not feel comfortable citing these findings as definitive conclusions. We have had to be pragmatic in our thematic analysis and while we hope these findings are a useful starting point for discussions about what 'well-led' services and organisations look like to those drawing on them, we believe they would benefit from further exploration, particularly from the perspectives of people drawing on care and support, family and friend carers, and members of the person-facing workforce.

Experiences of well-led services and organisations may be different in different national and local contexts.

This is alluded to within the available literature and was emphasised by lived experience partners during our co-production sessions. A well-led service or organisation may look different at a time of austerity (when achieving value for money and equitable spending might be all that it's possible to achieve) compared to a time of investment (when sincere efforts to invest in improvements would be expected). Lived experience partners reported the financial context to be the most important one to consider, as people drawing on care and support, family and friend carers, and families need to know that services and organisations are financially sustainable, and that they will have continuity.

Workforce shortages are also likely to have an impact, as are periods of political flux. There are also societal changes and 'public moods' that may impact what 'well-led' looks like, for example changing levels of trust in authority.

People may look for different attributes of services and organisations at times of crisis than in more static times. However, lived experience partners did not consider the COVID-19 pandemic to be a helpful model of flexibility and dispersed power, and suggest it's something of a red herring to keep focusing on it for the purpose of learning and improvement, as these quotes from separate participants show:

"Sure, some members of the workforce may have felt more empowered to get on with things and less likely to be sued, but people drawing on services often felt lucky to get anything, and family and friend carers often felt abandoned"

– Lived experience partner at one of this review's co-production workshops

"Yes, plus, it's questionable to say it empowered people to 'do the right thing', because 'the right thing' was so ambiguous and hard to determine. It was mostly 'get out as quickly as possible to avoid spreading infection', but for many people the right thing would have been to have a tea and chat and do their hair or paint their nails. You wouldn't want a service like that now, so what are we extolling?"

– Lived experience partner at one of this review's co-production workshops

“Also, we hear examples of people saying they didn’t have to deal with as much paperwork and they were empowered to make decisions, but unless we’re genuinely going to say, “All of that paperwork can be permanently scrapped”, then what are we saying and what is the point? Nothing in our current experience indicates health or social care would let go of their paperwork. It’s a hiding to nothing”

– Lived experience partner at one of this review’s co-production workshops

There may be no definitive description of what ‘well-led’ care and health services and organisations look like, as it may vary according to context. Though there may be key elements of what ‘well-led’ care and health services and organisations look like that persist regardless of context. These issues were not sufficiently addressed within this review to draw definitive conclusions.

It may be helpful to consider workforce values alongside the findings of this review. We did not explicitly consider values in this review. However, some values were repeatedly cited as important attributes of those who lead well-led services and organisations, including (but not limited to) kindness, humility, and honesty. These were felt to help guide the workforce within services and organisations to ‘do the right thing’ by those drawing on services. As we did not specifically seek to answer a question about values, we are cautious about overstating any findings. We recommend looking at TLAP’s work to consider values within the person-facing social care workforce alongside our findings and considering their implications for what ‘well-led’ means in health and social care.¹³

It can be tricky to distinguish experiences of ‘well-led’ services and organisations from experiences of ‘safe’, ‘effective’, ‘caring and/or ‘responsive’ services and organisations. There are also some things – like communication – that cut across all aspects of care and support. ‘Communicating in human’ is frequently cited in the literature as something expected of well-led services and organisations, but also as something that’s essential to achieving good, person-centred and coordinated care and support. This includes not using acronyms or labels, especially for people and groups of people (Sinclair & Farquharson, 2021a), avoiding language that is ‘othering’ (such as ‘vulnerable’) and that limits people’s power (Sinclair & Farquharson, 2020a), and avoiding language that puts the onus on people drawing on care and support, such as labelling them ‘hard to reach’ (Anandaciva, 2023).

¹³ For more information on TLAP’s work on values and the workforce visit: <https://thinklocalactpersonal.org.uk/resources/values-and-the-workforce-the-expectations-of-people-with-lived-experience/>

7. The findings

Cautious findings, of which we recommend further discussion:

A well-led service or organisation has a shared purpose that is focused on people drawing on care and support, family and friend carers, and families.

- Interactions with an organisation that has a purpose focused on people, feel different to interactions with an organisation that has a purpose focused on money or targets (Burton, 2015).
- People's positive interactions with a service or organisation should feel purposeful, and like good design, rather than like getting what you need through luck (Sutcliffe et al, 2015).
- If an organisation has a clearly stated purpose, then people can understand it and get behind it (McKenna, 2021).
- People [drawing on services] can challenge things they don't feel comfortable about or can push for more / new things by citing the shared purpose. For example, "You say you're about helping people stay independent but you're not doing that if you don't let me take risks so I can become more independent" (Manley, 2014).
- If an organisation has a clearly stated purpose, then people can understand it and get behind it (McKenna, 2021).
- People [in the workforce] notice what they're doing and think about it. They ask why they're doing what they're doing and if it's helping to achieve the shared purpose. They ask whose interests are served by it. And they ask whose interests are not served by it (Robertson, 2022).
- People [in positions of leadership] working in a service or organisation with a shared purpose can focus on making improvements that help to realise that purpose. A shared purpose helps prevent people being diverted / distracted by top-down diktats, structural changes, and so on. It provides some stability within a political, financial and policy context that's often in flux (Burton, 2015; NHS Horizons, 2018; Townsend, 2023).

Reflections from our co-production groups: This resonates with us. We agree with contributions that note compliance with regulations or with the achievement of targets (whether performance or financial) is not the same as giving the best care (for example, Burton, 2015). We also note that a shared purpose can help guide

people to 'do the right thing' and surmount condition-specific pathways or rigid processes so they can provide more holistic care and support.

A well-led service or organisation is guided by the lived experience of people drawing on care and support, their family and friend carers, and families.

- People are recognised as the experts of their own lives (Cwmpas, 2019; Slocock & Wyler, 2022; Leadership for Personalised Care, 2023).
- People are not asked to change their lives to meet the needs of the service; the service flexes around people's lives (Sinclair & Farquharson 2020a).
- People feel listened to, especially when they have concerns (Sutcliffe et al, 2015; Walls, 2021d; Townsend, 2023).
- People's strengths are recognised and built on (Sinclair & Farquharson, 2021a), but what people have lost is recognised too (Sorkin, 2023).
- People's fears are acknowledged and responded to. They are never belittled or suppressed (Robertson, 2023a).

Reflections from our co-production groups: We agree this is essential to achieving good, coordinated and personalised care and support. We note the importance of not only listening to people's lived experience but learning from it and then acting on it – both in terms of directly providing care and support and co-producing services and approaches – we need to move from “you said, we did” to “we said, we did – all together”.

We would like to see recognition that lived experience of a long-term condition or disability is not the same as lived experience of drawing on health and social care services.

There is something missing around cultural competence as a response to lived experience. A culturally competent organisation treats people of all races, cultures, religions, abilities, and orientations respectfully. Diverse perspectives are valued, and leaders act in ways that are fair to the entire team (Bosworth, 2022).

We recognise that people in the workforce have their own lived experience of drawing on services and of caring, and that this can be an asset (Robertson, 2023a). However, we feel this is a distinct point and have decided to include it within the next finding.

A well-led service or organisation has a culture of sincere curiosity and continuous learning.

- People [in the workforce, especially in positions of leadership] are curious and go into places / spaces that might make them feel uncomfortable. They never claim to know how to do 100% of their job. They admit when they don't know something, and they ask questions and learn from others. They encourage others to do the same (Robertson, 2023a).
- People [in positions of leadership within the workforce] find out what it is like to work in the organisation they're leading. They ask, "How does it feel to work here?", "What is it like for you?". People [in the workforce] feel comfortable they can give open and honest answers and that something positive will come of doing so (Robertson, 2023a; Skills for Care, 2023).
- People [in the workforce] are supported to find out about the wider context of what they do – the structures, ways of working, legislation, and so (McKenna, 2021; Messenger, 2022).
- People [in the workforce, especially in positions of leadership] are honest about what can be changed and what can't. They are honest about how long change will take. People feel cared about, even if it isn't possible to make the changes they'd like to see (Robertson, 2023a).
- People [in the workforce] expect to do the work around equality, diversity and inclusion. It is recognised that people with protected characteristics don't have the choice to duck out of the conversation for the day if they're not feeling it / don't have the energy / it feels a bit hard. It is recognised that asking people with protected characteristics to guide everything is draining. People show they are doing the work themselves (Anandaciva 2023).
- People are supported to learn about the people and communities they serve. This will likely mean moving along an educational journey, which in turn means everyone remembering where they started from. Shutting people down at the start of their journey means not allowing them to learn. It's important to create a safe space and allow people to sit in the space, even if it's uncomfortable, and ask questions and learn (Sinclair & Farquharson 2020b). This is sometimes referred to as having "clumsy conversations", which means people have a way of engaging that allows them to get things wrong and not feel judged or defensive (Preston & Samuel 2021).
- People feel confident that they can raise a concern, make a complaint or whistle-blow, and that something positive will come of doing so (Robertson, 2022). There is no blame culture (Messenger,

2022). People are rewarded for sharing what they learn after a mistake (Pereira et al, 2023).

- It is recognised that junior employees, because of the nature of their jobs, see issues and opportunities most immediately, and are most in touch with the actual rather than reported reality. It is recognised that people in the workforce who are more junior are less likely to speak up than those who are more senior, in-part because they have greater fears of being ignored or being suppressed (Reitz & Higgins, 2021).
- People feel success, contribution and accomplishment are consistently celebrated and learned from (Warren, 2022).

Reflections from our co-production groups: It is not possible to listen to, learn from and act on the lived experience of people drawing on care and support and family and friend carers unless that lived experience is actively sought out. However, a culture of sincere curiosity is also about understanding the experiences of members of the workforce: “People in positions of leadership need to be aware of and learn from the workforce’s understanding of what an organisation’s culture is – they mustn’t presume to know it” (lived experience partner at one of this review’s co-production workshops).

More exploration needs to be done to understand what a learning culture actually looks like in this context. For example, it’s not about curiosity and continuous learning in the sense of just going to conferences and speaking with other professionals – it’s about believing you can learn something from anyone, especially the people drawing on your service and wanting to listen and learn. This is where curiosity is often missing.

Our experience points to too much organisational effort going into reputational management instead of work to make things better. We believe a well-led organisation is one with an open culture where everyone is willing to admit mistakes. It is not defensive, where a no-blame, no excuse approach commits everyone to learning together proactively.

A well-led service or organisation is full of goodwill, but this adds value to what is offered, rather than holds things together.

- The workforce feels valued. This means that people are appropriately paid for what they do, and that they feel appreciated by the people they work with and for (Scott, 2015; Slocock & Wyler, 2022; Stacey & Sheth Trivedi, 2023).

- Core roles and responsibilities are delivered within the contracts people have and within the hours people are contracted to work (Scott, 2015; Stacey & Sheth Trivedi, 2023).
- People 'go above and beyond' because they want to and never because they feel they have to (for example to keep people safe). Their efforts are recognised and celebrated (Stacey & Sheth Trivedi, 2023).
- Change feels possible (Robertson, 2023a).
- People do not feel competitive within the workforce. "You do not need to blow out other people's candles to make yours burn brighter, but many do, and that sets the tone for the culture..." (Sabrina Cohen-Hatton, cited in Lawson 2019). Collaborative behaviour is encouraged and rewarded (Messenger, 2022).

Reflections from our co-production groups: This resonates for us, as we know people are proud to work for care and support organisations, including the NHS and to make a positive difference to people's lives, but we also know that this only goes so far. No one should have to work unpaid hours because they fear it is the only way they can make sure people receive good care and support or earn a living wage.

Goodwill feels like an underappreciated resource within health and care. It shouldn't hold the basics of service delivery together, but our experience suggests it often does. We also feel there needs to be frank consideration of the abuse of goodwill and its association with the abuse of relationships and the abuse of staff. We have seen staff having to go above and beyond because there would be safeguarding issues if they didn't, or they fear losing their jobs or being otherwise sanctioned. We know there are problems of modern slavery within health and social care, especially within the care sector (e.g., Holt et al, 2023). We personally know people-facing staff who have been traumatised by their experiences of supporting people with very high needs, of death, and of the emotional toll of this work. Well-led services and organisations by contrast, understand this and take care of their staff.

Acknowledging and appropriately utilising goodwill feels like something distinctive to well-led organisations. We can imagine scenarios whereby people looking at a service/organisation from the outside might see a lot of good work being done. They might conclude the service/organisation is 'Good' or even 'Outstanding'. In practice, the workforce might be on its knees, and the work only achieved through goodwill. The good work might even have been achieved despite the leadership rather than because of it.

A well-led service or organisation disperses power and builds trust.

- This is sometimes referred to as pursuing the principle of subsidiarity. The principle of subsidiarity means each decision is taken at the most local level that involves all the people and communities it affects (LGA, 2020).
- The principle of subsidiarity ensures: “there is clarity about which decisions are best made at a local level in order to respond to the needs of individuals and communities, and which decisions are best made at a more strategic level to achieve economies of scale; local communities are involved in making decisions about things that affect them; and decisions are made once, are empowering, accountable and transparent” (LGA, 2020).
- People trust that decision-makers aren’t going to “go rogue” and make decisions that make things harder (Vigor, 2023).
- Thin trust is turned into thick trust. Thin trust is, “You’ve done me no harm, but I don’t really know you yet” – it is quick to form but quick to break. Thick trust is much more evolutionary and has been built over time and/or experience – it’s not as fragile as thin trust and has more resilience to ups and down because of its roots / foundations (Robertson, 2022).
- Trust encourages open communication, enabling people who draw on care and support to express their concerns, fears, and desires without reservation. This, in turn, allows for the development of personalised care plans that address their unique needs (Farquharson, 2023).
- Trust allows people [in the workforce] to focus on their duties without unnecessary obstacles, ensuring that their efforts are dedicated to the betterment of the people they serve (Farquharson, 2023).
- It is understood that asking trust of people can make them feel vulnerable – “In trusting you I am vulnerable to you, I am trusting you will do what you say you will do, when you say you will do, and I am vulnerable if you don’t” (Robertson, 2022).

Reflections from our co-production groups: There’s some unpicking to do here, including what is meant by ‘power’ and how it relates to trust. The two concepts feel intuitively connected, for example, we are aware of scenarios whereby practitioners have been told “we’ve got your backs” by their leadership and have been supported and encouraged to take risks alongside people with lived experience – this is the sort of trust-based dispersal of power

that might be experienced within a well-led service or organisation. However, power has been found to decrease trust in social exchange, with people in low-power positions significantly more trusting of more powerful people than vice versa (Schilke, Reimann & Cook, 2015). There is more work needed to explore what power and trust look like in well-led services and organisations.

We also need to unpick whether people want power. Is the core issue about people being asked how much involvement they want in decision-making, or is it more than this? Is it different in health and social care? In social care, where the decisions are about people's day-to-day lives, the balance of power needs to be with the person. In health, this is a little different because more of the knowledge and skill is with the health professionals and therefore more power is ceded to them. This is where the link between power and trust is so important – if you are ceding power then you need to do so with trust. However, trust should be implicit even when you're not dispersing or transacting power – it's not power and then trust.

8. What do we suggest should happen next?

This review evidenced the need for TLAP to continue its work to develop 'well-led' as an extension of Making It Real.

Previously published evidence about what 'well-led' services and organisations look like has been heavily weighted towards the perspectives of senior staff and leaders within the workforce. Therefore, TLAP has been exploring what good leadership looks like for people drawing on care and support and family and friend carers, and people-facing members of the workforce.

So far, this has involved working alongside the Care Quality Commission (CQC) on a workshop series with people with lived experience and the health and social care workforce, to create a set of guiding principles that align with the areas of well-led within the CQC assessment framework ([Well-led – Care Quality Commission](#)).

TLAP is taking this work further to create a 'well-led' section for Making It Real, which will provide more information on what good leadership looks like within personalised care and support.

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